Ysbyty Gwynedd
Anaesthetics Department
Adran Anaestheteg

PERI-OPERATIVE

Anaesthesia for Major Gynaecological Surgery

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It is the responsibility of the prescriber to ensure that there are no contraindications to the following:

Premed:

'Pre-Op' carbohydrate drinks: x3 evening before and x3 on morning of surgery (can be taken up to 2 hours prior to surgery).

Anaesthetic:

GA [+/- Spinal with intrathecal diamorphine *0.5-1mg* if appropriate and non-laparoscopic surgery (10mcg/Kg **lean body weight** up to 80 years of age)* (unless contraindicated)

* Beware of an increased incidence of pruritus if using higher doses of intrathecal diamorphine, and prescribe naloxone & chlorphenamine PRN.

+ Abdominal wall regional anaesthetic blocks e.g. Rectus sheath block with catheters (surgeon to insert) for postoperative infusion (midline incisions only) or TAP block (4 quadrant if incision above T9)

Pre-incision:

Antibiotics (usually Gentamycin 240mg (160mg if <60kg) & Metronidazole 500mg, but check with surgeon and check for allergies)

Dexamethasone 0.1mg/kg

Intra-op:

Paracetamol 1g (or 15mg/kg if <50kg)

NSAID if no contraindications: (iv diclofenac 75mg or ketorolac 30mg **at the end** of surgery)

Opiate of choice as required, try to keep to minimum.

Ondansetron 4mg iv

Goal directed fluid therapy with SPV/PPV% or CardioQ

Optional: (Consider one or more of these)

- Magnesium Sulphate 5g (20mmol ampoule diluted in 50-100mls saline) infusion during surgery (started pre-incision) Titrate rate to HR and BP – *beware of hypotension & bradycardia*.
- Low dose Ketamine 0.15mg/Kg iv pre-incision *(NB Ketamine comes in 2 different concentrations. 100mg/ml concentration must be diluted – take care not to get them confused)*

Post-op: ***Call Acute Pain team (bleep 115) or Anaesthetist (063) if pain needs reviewing urgently***

Morphine PCA (1mg bolus, 5 minute lockout)

+ Rectus Sheath Infusion (8mls /hr of 0.125% Levobupivacaine via elastomeric pump) (for midline incisions)

Plus:

Regular:

Paracetamol 1g (or 15mg/Kg if <50Kg) QDS

Ondansetron 4mg iv / po TDS for 48 hours

NSAID (1st choice ibuprofen 400mg TDS) for maximum 14 days. *Reduce to 5 days in elderly. Avoid completely if history of ulcers (consider celecoxib 200mg BD); hypersensitive; significantly reduced GFR – see chart*

Omeprazole 20mg OD whilst taking NSAID

PRN:

Cyclizine 50mg tds slow iv, Naloxone 100mcg iv

Oramorph 10mg 2hourly when PCA taken down.

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To be prescribed by surgical team:

Clexane 40mg s/c in evening. *(20mg if <50kg, 40mg BD if >100kg)*

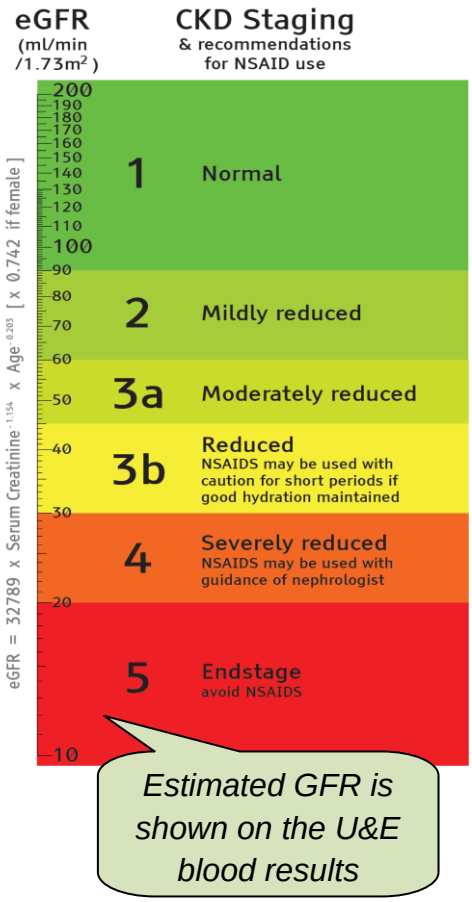
Consider: Senna 2 tablets BD plus lactulose 15mls BD

48 hours post op:

Continue paracetamol, NSAID, omeprazole, clexane, senna, lactulose as above

Codeine / tramadol either regular / PRN according to pain

Continue strong opiates if significant pain.



Estimated GFR is shown on the U&E blood results